

Direct Primary Care

A Physician-Owned Growth Model

Staggered associate coverage, protected clinical time
and a path toward practice ownership

An operating hypothesis for physician, operator and investor review

Physician development and patient access must support the same business

Local practice

One owner and two associates build continuing relationships. Membership funds care, follow-up and a defined response service.

Ownership pathway

Associates learn operations, mentor successors and may pursue ownership. Shared support must make practices easier to run at a fee they value.

The revised roster is a proposal to test. No recruiting interest, clinical outcomes, partnerships or funding commitments are assumed.

The proposed price needs evidence of a better patient experience

Established DPC category	Proposed variation
Periodic fees outside insurance billing	Core \$250 / Extended \$350 each month
Longer relationships and access	A staggered seven-day associate roster
Employment and ownership options	A mentored associate-to-owner pathway
Different local practice structures	Shared nonclinical support for a stated fee

Annual upfront: Core \$2,400 (\$200/month equivalent); Extended \$3,360 (\$280/month). Both save 20%. Paid demand is unproven.

Source: AAFP DPC overview and its 2024 data context. DPC Frontier definition. Proposed prices and service intensity are hypotheses.

Ten associate workdays cover seven days before leave or absences

Role	Sun	Mon	Tue	Wed	Thu	Fri	Sat
Associate A	9-9	9-9	9-9	9-9	9-9	Off	Off
Associate B	Off	Off	9-9	9-9	9-9	9-9	9-9
Owner*	Off	9-6	Off	9-6	Off	9-6	Off

9-9 means 9 a.m.-9 p.m. availability. Routine bookings end earlier. Each associate has a 12-encounter daily ceiling.

*Owner Monday/Wednesday/Friday is illustrative. All roles use 46 working weeks. Six weeks of absence per role still need relief and escalation coverage.

A twelve-hour window does not provide twelve one-hour routine bookings

Associate day, illustrative	Hours / capacity
Outer availability, 9 a.m.-9 p.m.	12 hours
Routine booking window, 9 a.m.-6 p.m.	9 hours
Breaks / messages / urgent reserve	1 hour each
Time left for routine appointments	6 hours
Routine capacity: all 60-minute / reference mix	6 / 6.86 visits per day

The 12 ceiling includes urgent and onboarding encounters. Three late hours remain response availability; onsite/remote arrangements are undecided.

Illustrative assumptions for expert review. No practice affiliation, operating data or commitments implied.

Weekday bottlenecks constrain enrollment before the monthly average does

Reference comparison	Earlier model	Revised roster
Routine capacity / month	299	320.4
Members within selected access target	363	325
Demand check	Monthly average	Daily weights + onboarding
Requested mature members	360	360

325 is an arithmetic planning limit, not a clinically validated panel. It includes 70% routine attendance, 85% utilization and replacement-member onboarding.

New pricing and base salaries improve the starting economics

Annual economics at 325 members	USD
Membership revenue at \$325 blended monthly dues	\$1,267,500
Physician pay plus employment costs	(\$576,000)
Existing local costs and platform fee	(\$270,000)
Result before coverage and bonuses	\$421,500
Evening/weekend support, relief, recruiting, extra operations	Unknown
Final result after bonuses	Incomplete

Base costs: \$846,000/year. Coverage costs and profit sharing are additional. Before tax, depreciation and financing.

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Booking windows and demand patterns change the enrollment limit

One change from the revised reference	Members within target
6 p.m. cutoff and reference weekday weights	325
Associates finish routine appointments at 7 p.m.	357
30-minute blocks with 25 physician minutes	434
Equal demand across all seven weekdays	219

Shorter blocks change the offering and require clinical review. Pricing does not increase the access limit. The 360-member request still exceeds the 325-member reference target.

\$150,000 base per associate,
plus quality-conditioned profit sharing

Annual-pay share	Revenue / year	Bonus / associate	Practice surplus
0%	\$1,267,500	\$35,125	\$337,200
50%	\$1,140,750	\$24,563	\$235,800
100%	\$1,014,000	\$14,000	\$134,400

20% of positive membership-funded operating surplus forms the total employer-cost pool. Bonus payroll burden is included; gross bonuses split equally among active associates/replacements.

Sensitivity: 325 members; 25% Core / 75% Extended; all four unpriced coverage increments set to \$0; all quality conditions achieved. No ancillary income. Final reference profit remains incomplete.

Reward care quality and continuity, with no per-sale physician commission

Labs and additional care

Document clinical need, disclose prices and handling fees, offer outside-provider choice. Separately price qualified nutrition or metabolic-health care only when it is outside the membership. No testing or prescribing quotas.

Peptides and evidence

Review each therapy individually for evidence, patient suitability, lawful sourcing and current status. FDA-approved medicines differ from unapproved or compounded products. Compounded drugs are not FDA approved.

Base case: \$0 ancillary income. Enter gross receipts and direct costs separately, and budget capacity. Ancillary revenue is excluded from the bonus basis. Terms require clinical and legal review.

Admissions follow available care as the team comes together

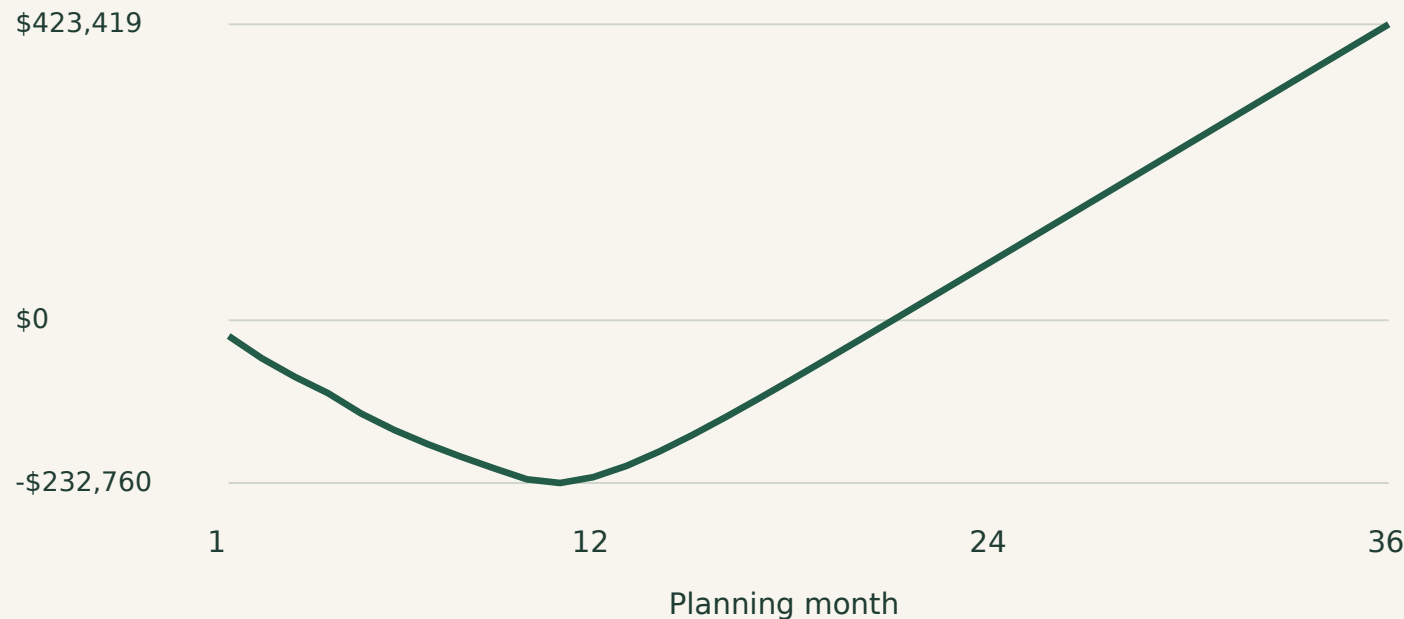
Planning input	Illustrative reference
Horizon / care opens	36 months / month 2
Opening applicants / later gross applicants	30 / 25 each month
Attrition / enrollment ceiling	1% per month / 360
Owner / Associate A / Associate B starts	Months 2 / 5 / 11
Actual modeled admission at opening	26.5 expected members

Before full staffing, the reference offers staffed-day access only. Unadmitted applicants wait without generating revenue. Their willingness to wait is unproven.

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The funding requirement stays incomplete until coverage and setup are costed

Diagnostic: cumulative operating result



\$423,419

36-month result after bonuses, with coverage increments set to zero. Sustained monthly operating break-even starts in month 12.

Coverage quotes, startup spending, opening cash and a reserve remain unknown.

This lower-cost sensitivity excludes startup spending, debt and financing. It is not a complete funding ask.

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Annual prepayments create an obligation while revenue is earned over time

Flow	Treatment
Annual payment at joining / renewal	Cash arrives immediately; revenue is earned monthly
Cancellation	Unused prepaid months refunded in this illustration
Startup spending	One cash outflow, separate from recurring costs
Opening cash / financing / reserve	Funding sources / funding source / target balance

Operating break-even, accumulated operating-loss recovery and startup investment recovery are different milestones. Financing is never operating revenue.

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Leadership time needs a funded reduction in clinical capacity

Proposed progression

Build patient relationships and operating judgment. Take on management and mentoring. Reduce appointments as agreed. Pursue ownership only with readiness, financing and continuity in place.

Modeled example

Associate A adds two management hours per working day from month 24, with unchanged pay. At that roster snapshot, the reference enrollment limit falls from 325 to 251 members.

The example is optional, not a promised timeline. Longer availability does not itself prove faster clinical development or readiness to lead.

A replacement adds expense before a departing physician can leave

Model the overlap

Enter a replacement start month, salary, working days and reduced initial availability. Deduct mentoring time from the trainer. A fourth paid physician during overlap is shown explicitly.

Protect the handoff

A departure removes that physician from this location. Retained members do not automatically transfer. If physical capacity falls short, the constrained cash forecast becomes incomplete.

A second location needs its own demand, startup and financing plan. Its earnings are not included in this one-location launch forecast.

Support-company fees must buy measurable value for practices

Equally mature locations	Practice revenue / year	Platform fees / year
1	\$1,267,500	\$60,000
25	\$31,687,500	\$1,500,000
100	\$126,750,000	\$6,000,000

Fees could fund scheduling systems, recruiting support, vendor coordination, reporting and owner development. Delivery costs and central overhead remain unknown.

Combined economics eliminate the internal fee transfer. Clinical authority stays with physicians. No investor return or exit multiple is assumed.

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A small set of tests determines whether the concept deserves capital

Hypothesis	Evidence / reviewer	Decision informed
Patients pay and stay	Paid demand and retention; patients + DPC operator	Price and included services
Work fits the roster	Visits, messages, urgent work, leave; physician lead	Panel, cutoff and coverage
Associates want the pathway	Recruiting discussions and terms; physicians	Pay, progression and ownership
Shared support earns its fee	Costed service trial; practice operators	Platform scope and margin
Launch can fund obligations	Quotes and cash scenarios; finance advisor	Funded pilot, then expansion

Practice cash flow and platform earnings are separate potential returns

Possible economic relationship

Physicians own and control clinical practices. Practices pay for agreed nonclinical services. A support company could be an investment vehicle if its contracts and earnings validate the case.

Decisions still open

Drew may build the support company or help a physician-led founding team. Founder roles, ownership percentages, buy-in terms, compensation and partnerships remain undecided.

Current legal and tax review must address clinical authority, service agreements, franchise questions and membership terms. No finalized structure or investment offering.

R06 preserves earlier inputs and makes the new assumptions editable

Review workflow

One open worksheet. Separate physician schedules and leadership stages. Add missing rows, leave unknowns blank, save locally and export a review file. Nothing is automatically sent.

Revision lineage

R01: initial concept and PDF. R02: comparison tool and green design. R03: independent DPC and launch model. R04: one worksheet. R05: staffing and daily constraints. R06: trial pricing, profit sharing and clearer coverage.

DPC-R06 / model dpc-2026-09-v5 / saved-file schema 5.

Reference finalized September 19, 2026 at 7:23 PM CDT (UTC-05:00). The web history links to release evidence.

Expert criticism should resolve coverage, economics and readiness

AAFP: DPC category, career options and price context

DPC Frontier: category definition

AMA: clinical and administrative work extend beyond appointments

IRS: distinct HSA contribution and reimbursement rules

Texas Medical Association: clinical-independence background

FTC: Franchise Rule guidance

Interactive model, full sources and revision history

Priority decisions: routine cutoff and response terms; coverage cost; patient-value evidence; ownership and succession terms.